

Surgeon Trust Accelerator

Building Documented APP Competency That Earns Full Autonomy

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The MBA PA

A framework for cardiac surgery programs to make trust visible, transferable, and systematic — replacing interpersonal guesswork with documented competency evidence that accelerates APP autonomy and survives surgeon turnover.

Why Surgeon Trust Breaks — And Why "More Time in the Room" Doesn't Fix It

The default approach to surgeon-APP trust is passive: put them in cases together, let the relationship develop organically, and assume competence will eventually speak for itself. Here's what actually happens:

THE DEFAULT APPROACH	WHAT IT PRODUCES
APP shadows for 4-8 weeks with no defined benchmarks	Surgeon develops opinion based on 2-3 memorable moments (usually errors), not overall trajectory
Competency determined by "when the surgeon says they're ready"	Different surgeons have different thresholds. Same APP — one signs off at month 3, another at month 9.
No documentation of specific procedure volumes or outcomes	When a new surgeon joins, the APP starts from zero — no transferable credibility
Trust assumed after a certain number of cases	Trust is withdrawn silently. One bad case can reset months of progress without documentation.

Surgeons build trust methodically: one procedure, one outcome, one repetition at a time. If your program doesn't mirror that methodical approach with documentation, you're always betting on personality instead of process.

1) The Procedure Autonomy Gradient

Define what trust looks like for every procedure — from observation to full autonomy.

LEVEL	DEFINITION	TRUST IMPLICATION	DOCUMENTATION REQUIRED
L0 — Observation	APP watches, may verbalize steps. No hands-on.	Zero trust. Baseline exposure.	Case log with procedure code and date
L1 — Partial Participation	APP performs discrete, low-risk sub-tasks under direct supervision (skin closure, scope holding, branch ligation)	Trust in isolated components	Procedure-specific checklist with sub-task sign-off
L2 — Supervised Full	APP performs entire procedure with attending scrubbed, ready to intervene	Trust in technical execution under oversight	Full procedure evaluation form with attending signature
L3 — Independent / Available	APP performs independently; attending in OR suite but not scrubbed unless called	Trust in independent judgment and technical execution	Independent procedure log with outcome notation
L4 — Full Autonomy	APP performs without attending presence; attending available by phone	Complete trust. APP is procedure owner.	Autonomy sign-off with surgeon attestation

Implementation: Map every procedure your APPs perform — not just the big ones. Include the "small" procedures where trust erosion often hides: sternotomy closure, chest tube insertion, central line, arterial line, TEE probe positioning. For each, define L0-L4 specifically. "Independently harvests vein" is not specific. "Independently performs endoscopic saphenous vein harvest from incision to conduit handoff, including CO2 management, branch ligation, thermal spread prevention, and standardized handoff with documented conduit grade" — that's specific.

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Competency Documentation & Trust Reset Protocol

2) The Individual APP Competency Dashboard

Make progress visible in a format surgeons recognize as credible. Surgeons trust data. Give them data.

Living Competency Dashboard

PROCEDURE	LEVEL	LAST ADV.	CASES	TARGET	TO GATE
Endoscopic SVH	L3	2026-05-12	34	L4	16
Radial harvest	L2	2026-04-28	11	L3	9
Sternotomy closure	L4	2026-01-15	62	L4	N/A
Central line	L4	2025-11-03	89	L4	N/A

3) The Gate Advancement Packet

When an APP is ready to advance — hand the surgeon evidence, not a gut check.

Gate Packet Contents

1. ● **Case log:** Every procedure at current level, with dates and attending names
2. ● **Outcome summary:** Complications, rejections, take-overs, notable events — full history, not cherry-picked
3. ● **Preceptor evaluations:** Structured feedback from each surgeon the APP has worked with
4. ● **Self-assessment:** APP's own reflection on readiness, strengths, and areas for continued development
5. ● **Advancement recommendation:** "Based on [X] cases with [Y] outcomes and [Z] preceptor sign-offs, advancement recommended effective [date]."

4) The "Trust Reset" Protocol

When trust is withdrawn — document it like any other clinical event.

Trust Reset Documentation

- ☐ **Event description:** What happened? Objective, no blame language.
- ☐ **Trust impact:** What specifically is the surgeon no longer comfortable with? (e.g., "No longer comfortable with independent radial harvest" — not "lost trust in APP")
- ☐ **Return-to-trust path:** How many supervised cases at what level before re-evaluation?
- ☐ **Timeline:** When will re-evaluation occur?
- ☐ **Surgeon sign-off:** The affected surgeon agrees to the return-to-trust criteria.

Trust resets are normal in high-stakes clinical environments. What's abnormal is leaving them undocumented and unresolved.

5) New-Surgeon APP Briefing — Transfer Trust, Don't Rebuild It

When a new surgeon joins, hand them evidence — not an awkward introduction.

Briefing Document Contents

1. ● **APP roster with competency dashboards:** Every APP, every procedure, current autonomy level, case volumes
2. ● **Program trust standards:** "In this program, an APP advances to L3 after [X] supervised cases with [Y] outcomes."
3. ● **Escalation protocol:** "If you believe an APP's autonomy should be adjusted, here's how — a documented trust reset with defined return criteria."
4. ● **Preceptor expectations:** "As an attending, you're expected to complete a brief evaluation after each supervised case. This builds the evidence base."

6) 90-Day Trust Transfer Plan for New Surgeons

WEEK	FOCUS	OUTPUT
1-2	Surgeon observes all APPs at documented autonomy levels	Confirms or flags discrepancies vs. observed performance
3-4	Surgeon works primarily with L4 APPs on complex procedures	Calibrates to program standards for "full autonomy"
5-8	Surgeon supervises L2-L3 APPs with documented competency packets	Adds evaluation data to existing APP records
9-12	Full integration — program-standard autonomy assignments	Outstanding trust flags resolved or converted to formal protocols

7) Trust Metrics Dashboard

Track what surgeons actually care about: outcomes, efficiency, reliability.

Quarterly Trust Metrics

METRIC	DEFINITION	GREEN	YELLOW	RED
Time to L3	Median weeks from hire to independent status on core procedures	≤16 wks	17-24 wks	>24 wks
Trust Score	Quarterly anonymous surgeon survey (1-5)	≥4.5	3.5-4.4	<3.5
Autonomy Utilization	% of cases where APPs operate at documented autonomy level	≥90%	75-89%	<75%
Trust Resets	Formal trust reset protocols activated per quarter	0	1	≥2
Reset Resolution	Median weeks to return-to-trust sign-off	≤4 wks	5-8 wks	>8 wks
New Surgeon Transfer	Median weeks for new surgeon to confirm existing APP autonomy levels	≤4 wks	5-8 wks	>8 wks

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Program Self-Assessment & Implementation Toolkit

8) Self-Assessment — Personality or System?

Rate your program across 7 dimensions of trust infrastructure.

Seven Dimensions of Trust System Maturity

1 Procedure autonomy levels formally defined

0 pts — No levels. 1 pt — Informal expectations. 2 pts — Every procedure has documented L0-L4 criteria. Score: ____ / 2

2 Competency documented with case logs and outcomes

0 pts — Not documented. 1 pt — Incomplete logs. 2 pts — Living dashboards updated monthly. Score: ____ / 2

3 Advancement uses structured gate packets

0 pts — "Surgeon says ready." 1 pt — Some documentation. 2 pts — Full packet: case log, outcomes, preceptor evals, self-assessment. Score: ____ / 2

4 Trust resets have documented protocol

0 pts — Informal. 1 pt — Verbal criteria. 2 pts — Written protocol with return path, timeline, surgeon sign-off. Score: ____ / 2

5 New surgeons receive APP competency briefing

0 pts — Nothing. 1 pt — Verbal overview. 2 pts — Written briefing with individual APP dashboards. Score: ____ / 2

6 Surgeon trust systematically measured

0 pts — Never measured. 1 pt — Informal feedback. 2 pts — Quarterly anonymous survey with trended data. Score: ____ / 2

7 Autonomy utilization systematically tracked

0 pts — Not tracked. 1 pt — Anecdotally aware. 2 pts — Measured and reviewed quarterly. Score: ____ / 2

Composite Score: ____ / 14

10–14 Systematized. Trust transfers between surgeons. New APP integration is accelerated. Trust disruptions have defined recovery paths.

5–9 Developing. Pieces in place, but the system lives in your APP lead's head. If they leave, trust infrastructure leaves with them.

0–4 Personality-Dependent. Trust is a function of individual relationships. One new surgeon, one APP departure, or one bad outcome can destabilize everything.

What's Inside the Full Surgeon Trust Accelerator Toolkit

- **Procedure autonomy gradient templates** — L0-L4 definitions for 15+ cardiac OR procedures
- **APP competency dashboard template** — spreadsheet-ready, presentation-ready, surgeon-review-ready
- **Gate advancement packet template** — case log, outcome summary, preceptor evaluation, recommendation letter
- **Trust reset protocol template** — event documentation, return criteria, timeline, surgeon sign-off
- **New-surgeon APP briefing template** — APP roster, dashboards, program standards, escalation protocol
- **Quarterly trust metrics dashboard** — pre-built with green/yellow/red zones and trend tracking
- **Surgeon trust survey instrument** — anonymous, validated, ready to deploy

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This is an operational systems framework, not clinical teaching or patient-care directives. It draws on established organizational psychology and surgical education principles applied to cardiac surgery team management.

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